

WHAT INPURSUIT IS

InPursuit is the *infrastructure* for person-centric clinical services.

Not a platform. Not a SaaS. Not another wellness app.

The data-orchestration layer the consumer app, the partner-practice network, and the concierge service all run on.

330M

AMERICANS

230K

SMALL & MID-SIZE PRACTICES

\$365B

CONVERGING U.S. MARKETS

DIGITAL HEALTH • CARE MGMT • CONCIERGE
• WELLNESS

InPursuit Health

1 INFRASTRUCTURE DELIVERING
VALUE TO ALL 3

HEALTH & WELLNESS APPS HAVE FAILED

Both Consumers and Providers have been waiting for the *same fix*.

IF YOU'RE A CONSUMER OR PATIENT

\$1,092_{/yr}

Spent on digital health & wellness apps. *McKinsey Wellness Consumer Report, 2025.*

3–7 Apps

On the average person's phone — none talking to each other.

HARNESS THE POWER OF YOUR DATA™

"Stop feeding the machine. Start owning your health."

IF YOU'RE A PROVIDER

500M₊

Patient portal app downloads — cumulative across MyChart, healow, HealtheLife, FollowMyHealth, athenaCommunicator.

76% Gone

Open the app once — never come back. Engagement isn't satisfaction. It's a revenue strategy.

RESULTS MATTER™

"Start generating better outcomes."

THE SUPPRESSED SIGNAL

83.1%

Concierge & DPC growth in 5 years. *Health Affairs, 2025.*

\$38B_{by 2031}

Concierge medicine market — growing 9% CAGR. The fastest-growing segment in American healthcare is the one built to escape it.

CONCIERGE FOR EVERYONE™

"Premium care. Finally accessible."

Two broken experiences. One emerging escape. *One infrastructure that delivers value to all three.*

WHY NOW · PROVEN & UNTAPPED

A proven and *untapped* market.

Four large, fast-growing markets are converging on the same patient — the model is already funded by others’ billions, and the field is still wide open.

U.S. MARKET CONTEXT

Where InPursuit *plays*.

Each large · each growing · converging on the same patient.

DIGITAL HEALTH (U.S.)

\$92B → \$266B

11.2% CAGR · 2025–2035

RPM · CCM · CARE MGMT

\$14.7B → \$33.3B

14.8% CAGR · 2024–2030

CONCIERGE MEDICINE (U.S.)

\$8.1B → \$21.6B

10.3% CAGR · 2025–2035

CORPORATE WELLNESS (U.S.)

\$23B → \$44B

6.6% CAGR · 2026–2035

CONSUMERS ARE PULLING

32%

of U.S. adults now use an **AI chatbot for health information** — **doubled in a year** (16% in 2024). *Rock Health 2025, n=8,000+.*

53%

own a **connected device** and track at least one health metric digitally.

DIGITAL HEALTH ADOPTION

MAINSTREAM

HARNESS THE POWER OF YOUR DATA™

"The demand is already here."

PROVIDERS ARE OUTSOURCING

\$29_B

U.S. remote-care market by 2030 — ~\$16B today, growing **12.6% CAGR**. *MarketsandMarkets, 2026.*

\$6_{B+}

Commure (Athelas merger) — the category’s funded leader; Cadence is a unicorn. **Billions already proved practices will outsource.**

THE MODEL

VALIDATED

RESULTS MATTER™

"Funded — and barely penetrated."

THE WHITE SPACE

~4%

of eligible Medicare patients actually **receive chronic care management** — though ~3 in 4 qualify for it.

~12K

of the ~1 million Medicare providers bill CCM at all. Leaders combined reach **thousands of 230K+ practices**. *Mathematica / KFF, 2024.*

ELIGIBLE PATIENTS REACHED

<4%

THE OPPORTUNITY

"Proven, paid for — and barely touched."

Proven by others’ billions. *Untapped by all of them.* We enter a category that already works — and that no one has captured.

WHY NOW • THE \$50B RURAL FUND

\$50 billion to the states. *We’ve already filed in the first one.*

The Rural Health Transformation Program is live in all 50 states — ~\$200M average first-year award, every dollar obligated by October 30, 2026. That deadline is pushing money to providers now. We target where an open window meets a rural provider we can apply through.

\$50B

OVER FIVE YEARS

~\$200M

AVG FIRST-YEAR AWARD

50 states

LIVE NOW

Oct 30 2026

OBLIGATION DEADLINE

TIER 1 • OPEN NOW • ACTIVE APPLICATION

Mississippi

\$205.9M first-year award

Honeycomb — directly eligible applicant

InPursuit — named technology partner

Submitted awards projected August 2026

RTG pool: \$200K–\$400K per project

First revenue and the repeatable proof start here.

TIER 2 • NEAR TERM

North Carolina	HOME STATE	\$213.0M
Alaska		\$272.2M
Tennessee		\$206.9M
Wisconsin		\$203.7M
South Carolina		\$200.0M
Oregon		\$197.3M
Minnesota		\$193.1M
Idaho		\$186.0M

Windows live or imminent — each needs a rural in-state applicant.

TIER 3 • BUILD PIPELINE

Georgia	\$218.9M
Kentucky	\$212.9M
Arkansas	\$208.8M
Louisiana	\$208.4M
Alabama	\$203.4M
Texas	large-state

Line up a rural in-state lead first, then apply.

HOW WE CHOOSE an open window • the right program (technology, not workforce-only) • provider access • rural density. State stages shift weekly; each is verified against the state’s own posting before we act.

WHY NOW • MARKET CONTEXT & CONVERGING TAILWINDS

This isn't a trend. *It's an inflection point.*

01 FEDERAL POLICY • MAHA

Genie's Out

The patient-centric ecosystem is now **structural federal policy**.

CMS's Health IT Ecosystem and MAHA are **tearing down digital walls** — returning data ownership to patients, breaking the EHR monopoly, and unlocking AI-billable clinical services. **This doesn't reverse with the next administration.**

CMS • WHITE HOUSE "MAKE HEALTH TECH GREAT AGAIN" • JUL 30, 2025

02 CONSUMER DEMAND

The Shift.

73% want control of their own health data — not reliance on provider portals.

75% want personalized recommendations. **Only 34% want their next visit to be in a doctor's office.** When they do go in person, they want **trust and bedside manner** — not factory medicine from a large system.

VERILY/HARRIS POLL JUL 2025 • PWC HEALTH CONSUMER SURVEY 2025

03 INDEPENDENT PROVIDERS

The BOOM.

76% of physicians say AI improves their ability to **deliver better patient care** — up from 65% in 2023.

94% are **using AI or interested in adopting it**. 2026 PFS codes flow directly to their NPI — not the hospital's. The 230K small & mid-size practices that held the line are **ready for the digital infrastructure to scale** — and the new charge codes to follow.

AMA 2026 AI SENTIMENT SURVEY (N=1,700) • DOXIMITY 2026 STATE OF AI (N=3,151) • CMS 2026 PFS FINAL RULE

04 ALL PAYERS

They're Paying.

Every payer category is moving money to **outcomes, not volume**.

CMS leads — APCM, CCM, RPM, TCM concurrently billable in 2026. **MA parity** mandates Medicare Advantage match Medicare. **Commercial follows** — CCM/RPM now span nearly all major payers. **Employers prioritize** virtual-care integration. The whole stack is paying for better outcomes.

2026 CMS PFS FINAL RULE • BUSINESS GROUP ON HEALTH 2026 • HCPLAN APM MEASUREMENT

WHAT INPURSUIT IS • HOW WE DELIVER IT

InPursuit is not just an app. *It is the infrastructure for personalized health & wellness — supporting both consumers and providers.*



ENGINE 01 • D2C APP MEMBERSHIP

The InPursuit App

A consumer membership that gives every American ownership of their health data — and pays them back for using it.

HARNESS THE POWER OF YOUR DATA™

ENGINE 02 • THE PARTNER NETWORK

The InPursuit Partner Network

The infrastructure that lets independent practices offer digital clinical services they couldn't deliver on their own — and get paid for them.

RESULTS MATTER™

THE CONSUMER SIDE

330_M

Americans — finally able to own and use their own health data

80% favor **remote monitoring** of their health by their care team.

NATIONAL SURVEYS, 2022-2024

THE PROVIDER SIDE

230_K

Small & mid-size practices — finally able to offer digital clinical services

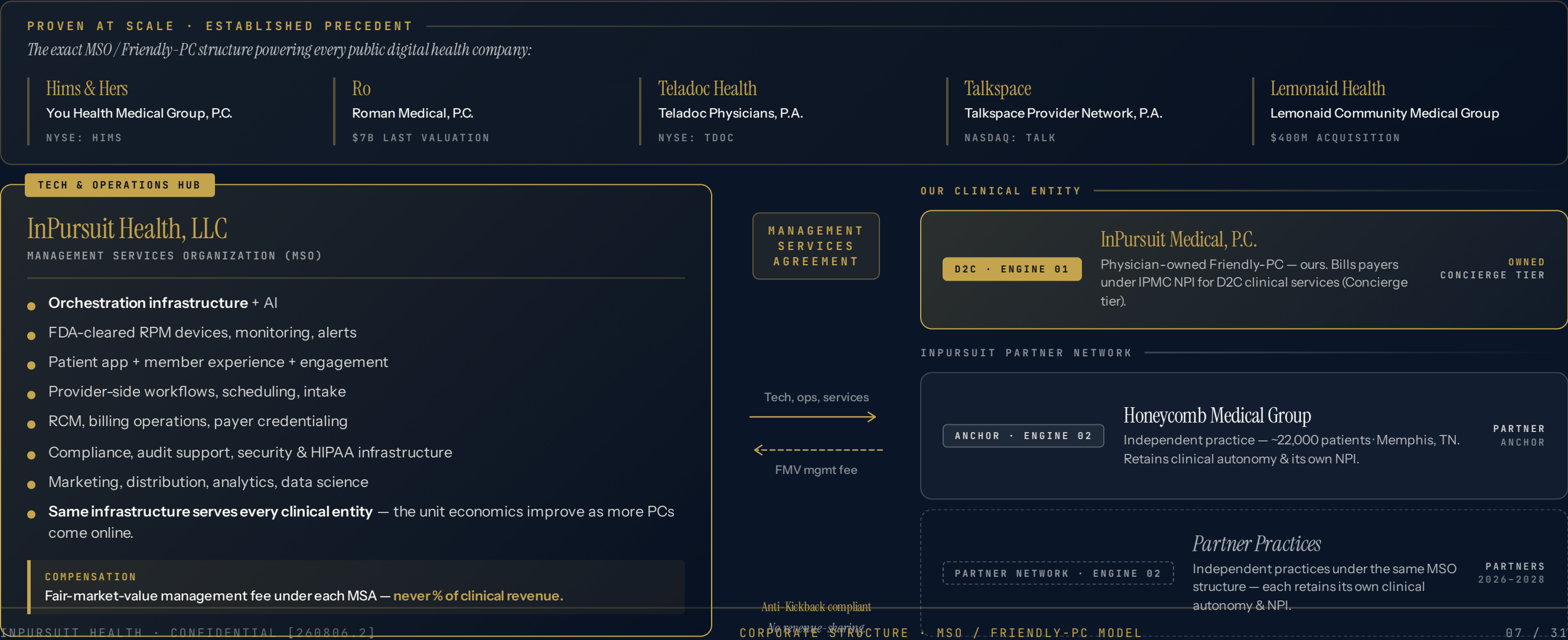
65% of patients would **switch doctors** for a better digital experience — up from 55% the year before.

TEBRA PATIENT PERSPECTIVES REPORT, 2025

INPURSUIT HEALTH · CORPORATE STRUCTURE

The same structure as *every successful digital health company*.

One MSO. Multiple clinical entities. InPursuit Health is the technology & operations infrastructure underneath every clinical relationship — D2C through InPursuit Medical, partner practices through the Partner Network. It's the proven model that built Hims, Ro, Teladoc, Talkspace, and Lemonaid. *Representative structure; the actual entity and billing configuration is established state by state to conform with each jurisdiction's corporate-practice-of-medicine requirements.*



INSIDE ENGINE 02 · BILLABLE CLINICAL SERVICES

The InPursuit app becomes *a billable clinical surface*.

Once InPursuit Medical PC is credentialed, the same app that delivers Concierge becomes a **federally reimbursable clinical infrastructure** — billed to Medicare, MA plans, and commercial payers under IPMC’s own NPI.

PREREQUISITE

NPI obtained + payer credentialing complete. Billing capability activates per state as credentialing finalizes. Entity and billing configuration is established state by state to conform with corporate-practice-of-medicine requirements.

Remote Patient Monitoring FDA-cleared connected devices, NP-led review, and patient outreach delivered through the app. 99454 Device supply & transmission \$52.11/mo 99457 First 20 min mgmt \$51.77/mo MEDICARE · MA · COMMERCIAL

THE VALUE · FOR BOTH OF YOU

Here’s what changes *for both of you.*

THE STRUCTURAL DIFFERENCE

InPursuit is a *membership*, not a subscription. And unlike a subscription, *it pays you back.*

FOR YOU

What changes *for you.*

HARNESS THE POWER OF YOUR DATA™

TODAY

Every provider holds a piece of your health. **You hold none of it.**

WITH INPURSUIT

You hold all of it — one record, one device, plain language.

TODAY

Your data generates billions in value — **none of it returns to you.**

WITH INPURSUIT

Your data returns value **back to you** — savings, cashback, care.

TODAY

Big Tech trains AI on your health data — without your consent.

WITH INPURSUIT

AI works **on your data, on your terms.**

*You stop being the customer of your healthcare.
You become the owner of it.*

FOR YOUR PRACTICE

What changes *for your practice.*

RESULTS MATTER™

TODAY

No infrastructure for coordination, RPM, or CCM — **revenue left on the table.**

WITH INPURSUIT

RPM, CCM, TCM, BHI — delivered through our infrastructure.

TODAY

Value-based care is a tax — **you take the risk, can’t build the infrastructure.**

WITH INPURSUIT

The infrastructure is **already built.** You just see the patients.

TODAY

Margins shrink yearly as commercial reimbursement **compresses.**

WITH INPURSUIT

A **second revenue line** — growing independent of fee-for-service.

*You stop being squeezed by the system.
You start being paid by it.*

ENGINE 01



The D2C InPursuit Health *App.*

Harness the Power of Your Data™

ENGINE 01 • D2C APP MEMBERSHIP

Harness the Power of *Your Data.*TM

A universal, all-payer app on Apple App Store, Google Play, and the Medicare App Library — one membership that provides *personalized care and wellness, regardless of what you are InPursuit of.*

01 • PERSONALIZED

Care built for *you.*

Your record, your data, your wellness goals — **guidance tailored to your life.**

02 • SAVE MONEY

Spend less, *get more.*

Hidden savings on Rx, procedures, and benefits you didn't know you had — **\$5K+ avg/yr.**

03 • SAVE TIME

Hours back, *not lost.*

No more portal logins, hold music, or paper forms — **42+ hrs recovered/yr.**

04 • YOUR FEED

A living health *feed.*

New insights, content, and offers added **continuously** — not a one-time download.



Robert, 58
TYPE 2 DIABETES • MEDICARE

IN PURSUIT OF

Better A1C, fewer complications.

Walks, meds, glucose readings, and lab trends in one view. AI flags when his morning numbers drift — not just at the next visit. **His PCP bills CCM & RPM through Medicare** with InPursuit's telemetry as the source of truth.

- HEALTH** A1C trend visible weekly • med adherence up
- MONEY** Same metformin: **\$47 → \$4** via Costco / Humana formulary
- TIME** 3+ hrs/mo on calls and portal logins — gone



Sarah, 32
NEW MOTHER • CIGNA PPO

IN PURSUIT OF

A family that grows with InPursuit.

Pregnancy → delivery → her baby's pediatrician — every record, milestone, and cost in one feed. **The Family tier (\$34.99) brings her spouse and baby on day one.** Postpartum BHI billed by her OB. Pediatric attach surfaces automatically.

- HEALTH** Every prenatal milestone tracked • postpartum BHI screen done
- MONEY** Delivery: **\$4,200 vs \$8,900** — same in-network NICU level
- TIME** One record across OB, pediatrician, PCP — no retelling



Elena, 67
BREAST CANCER SURVIVOR • MEDICARE

IN PURSUIT OF

Survivorship without surveillance gaps.

Three years post-treatment. **Oncology surveillance, cardiology follow-up, and Medicare wellness visits** coordinated in one record. Lymphedema flag, cardiotoxicity monitoring, and bone-density tracking all feed her care team in real time.

- HEALTH** Recurrence-watch protocol on-time • cardio risk caught early
- MONEY** **\$0 copay** on CCM & RPM — Medicare-covered through her PCP
- TIME** No re-faxing records between oncologist, PCP, cardio

\$50/mo in → \$400–\$700/mo back

D2C MEMBERSHIP · TWO LADDERS, TWO FLAGSHIPS

Two ladders. *Two flagships.* One infrastructure.

An individual ladder and a household ladder, each with its own intelligence flagship and gated clinical capstone. The intelligence layer always gates the clinical layer — a clinician without it is a commodity.

ARCHITECTURAL RULE

Intelligence gates clinical.

Concierge sits exclusively on **Total Intelligence**; Family Concierge sits exclusively on **Total Family Intelligence**. The clinical relationship is never sold standalone — with the intelligence layer underneath, it becomes a category.

GATED TO TOTAL INTELLIGENCE

ADD-ON

Concierge

+ \$29 /mo

IPMC CLINICAL MEMBERSHIP

A real clinician working from your data. Same-day async, **2 telehealth visits/mo**, Rx continuity. The front door to IPMC.

GATED TO TOTAL FAMILY INTELLIGENCE

ADD-ON

Family Concierge

+ \$49.99 /mo

Up to 4 members · +\$12/mo per additional

IPMC CLINICAL MEMBERSHIP

Household clinical access via **InPursuit Medical, PC**. Async, telehealth, prescription continuity.

↑ REQUIRES TOTAL INTELLIGENCE

↑ REQUIRES TOTAL FAMILY INTELLIGENCE

TOP OF FUNNEL

Free

\$0 /mo · forever

Your fragmented health record, finally in one place.

- TEFCA / Carequality record aggregation
- Baseline Reflect facial scan
- Basic alerts & reminders

NO CLINICAL

THE REFLECT HUB

Plus

\$14.99 /mo

Every device you own, finally talking to each other.

- Wearables, CGMs, BP cuffs, scales
- Sleep trackers · third-party apps
- Contextualized to your record

NO CLINICAL

HOUSEHOLD

Family

\$34.99 /mo

Up to 4 · +\$8/mo per add'l

Plus, for the whole household.

- All Plus features for every member
- Children, spouses, aging parents
- One app for the whole family

NO CLINICAL (BASE)

INDIVIDUAL FLAGSHIP

THE INTELLIGENCE LAYER

Total Intelligence

\$49.99 /mo

Data into decisions — the gate to a clinician who knows you.

- Predictive risk · AI insights
- Rx pricing · price transparency
- In-network search · HSA/FSA
- Outcomes dashboard · navigation

GATES CONCIERGE

HOUSEHOLD FLAGSHIP

HOUSEHOLD INTELLIGENCE

Total Family Intelligence

\$89.99 /mo

Up to 4 · +\$8/mo per add'l · \$22.50/person at four

The full intelligence layer — for the entire household.

- Everything in Total Intelligence
- Extended to every household member
- ~55% below per-head pricing
- The SKU employers sponsor

GATES FAMILY CONCIERGE

INSIDE ENGINE 01 • THE D2C CONVERSION ENGINE

InPursuit Reflect™

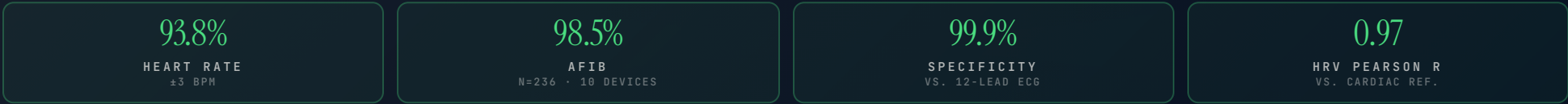
How InPursuit converts D2C members from Free to Plus — and earns the right to deliver clinical services.



FACE REFLECT™ • 50-SEC SCAN
ON-DEVICE • NOTHING UPLOADED

21 Metrics. One Scan. Clinical-Grade Accuracy.

7 FDA-CLEARED • 4 CLINICALLY VALIDATED • 10 WELLNESS & RISK SIGNALS



Outperformed Apple Watch ECG & leading 6-lead accessories in 2025 head-to-head — **5.6%** insufficient reading rate vs. **13.5%** Apple Watch.

D2C ENGAGEMENT ENGINE • REFLECT PRICING

Free

\$0 / mo

10 scans / mo

Try it. Keep your record.

Plus

\$14.99 / mo

Up to 300 / mo

+ wearable sync • trends • alerts

UNLOCK THE HUB

Every Wearable. Every App. *One Place.*
REFLECT IS THE BIOMETRIC HUB — YOUR SCANS + EVERY
DEVICE YOU ALREADY OWN.

21+
INTEGRATIONS
AT LAUNCH

- Apple HealthApple WatchOura RingGarminWhoop
- FitbitSamsung HealthGoogle FitWithingsDexcom
- Polar... and more

Why this matters strategically: Every scan deepens the member relationship and feeds the InPursuit record. *Free converts to Plus.* Plus members scan daily — **that’s the data Engine 2 clinicians (IPMC) need to bill CCM, RPM, BHI.** Reflect is how InPursuit earns the right to deliver clinical services D2C.

INSIDE ENGINE 01 • THE PREMIUM TIER

The clinical relationship sits *on top of the intelligence layer*.

Forward Health charged \$149/mo for a doctor without the data — and shut down. We do the opposite. Members build the intelligence layer first at \$49.99/mo. Concierge then adds an IPMC clinician on top for \$29 more. The intelligence is what makes the clinician differentiated.

INPURSUIT CONCIERGE

A clinician working *from your data*.

Async messaging, same-day virtual visits, prescriptions, referrals — with a clinician who can see every device, every lab, every Rx, every alert. No employed PCP in America has this context.

Total Intelligence (\$49.99) carries the intelligence stack. Concierge (\$29 add-on) adds the InPursuit Medical PC clinical relationship. Together: **\$78.99/mo for a clinician who knows everything.**

- NP-led pooled clinical panel
 - 2 telehealth visits/mo included
 - Rx, labs, referrals managed
 - Engine 2 clinical assessment at onboarding
- Async messaging, same-day SLA
 - Prescription management
 - Continuity across unified record
 - Pathway to CCM, RPM, BHI billing

“The intelligence layer is what makes 15 minutes of clinician time deliver more than a 30-minute visit at One Medical.”

THE STRUCTURAL UNLOCK

Concierge is *the front door to Engine 2*.

Concierge enters members into the InPursuit Medical PC clinical relationship. From there, **CCM, RPM, BHI, and TCM** are billed by IPMC to Medicare and commercial payers — **independently of the \$29 subscription**. The line item is a strategic loss; the bundle plus Engine 2 conversion is where margin lives.

ENGINE 2 • SAMPLING

What IPMC bills *when Concierge converts*.

99490	CCM	Chronic care, 20 min	\$66.30/mo
99457	RPM	Remote mgmt, 20 min	\$51.77/mo
60557	APCM	Primary care, 2+ chronic	\$53.91/mo
99484	BHI	Behavioral health, monthly	~\$48/mo
60439	AWV	Annual wellness visit	~\$138/visit

2026 CMS PFS FINAL RULE (CMS-1832-F) • NATIONAL AVERAGES • ALL BILLED UNDER IPMC’S OWN NPI

\$78.99/mo EFFECTIVE BUNDLE PRICE (TI + CONCIERGE)	\$-23.14 CONCIERGE LINE-ITEM MARGIN (STRATEGIC)	+\$42.50 ENGINE 2 UPLIFT PER CONCIERGE MEMBER	\$50.95 TRUE BUNDLE MARGIN (INCL. ENGINE 2)
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INSIDE ENGINE 01 · CASH-PAY CLINICAL · VS RO & HIMS

Ro and Hims charge \$149 for the membership. *Ours is \$79 — on top of everything.*

Same category, roughly half the fee — and a clinician who actually has the member’s data. Ro and Hims bill \$149/mo for the telehealth wrapper alone; medication is separate, all-in runs \$300–\$600. Our program rides on the Total Intelligence + Concierge layer the member already carries.

RO · HIMS — STANDALONE

\$149_{/mo}

membership alone — medication billed separately

+ \$149–\$399/mo medication

≈ **\$300–\$600/mo all-in**

Async text · single condition

No unified record · no continuity

RO & HIMS PUBLISHED PRICING · JUL 2026

INPURSUIT — ON THE INTELLIGENCE LAYER

\$79_{/mo}

program — on the \$78.99 TI + Concierge base

\$158/mo all-in — **with the full intelligence stack**

Concierge clinician from your complete record

Every device, every lab · unified continuity

56% contribution margin

HALF THE FEE · THE WHOLE INTELLIGENCE LAYER

ONE MODEL · THREE PROGRAMS

Each billed on top of Total Intelligence + Concierge (\$78.99).

GLP-1 / WEIGHT	\$79 _{56%}
MEN’S HEALTH	\$79 _{51%}
WOMEN’S HEALTH	\$59 _{56%}

The clinical program the member already wanted — captured on infrastructure that’s already paid for.

~\$1.61M cash-pay line

~1,765 members · ~1% of base

~55% blended contribution

Intelligence gates clinical — the layer Ro and Hims can’t build.

CMS HEALTH IT ECOSYSTEM · FIRST WAVE PARTICIPANT · APRIL 9, 2026

Three channels. 67million Medicare lives. A front door most pitches don’t have.

First-Wave selection into the CMS Health IT Ecosystem opens a direct distribution path to every Medicare beneficiary in America — the highest-value cohort in D2C digital health, reached through a CMS-promoted channel — not cold paid media alone.

APRIL 9, 2026 · WHITE HOUSE LAUNCH

CMS Health IT Ecosystem · *First Wave Participant*

Selected alongside Amazon, Apple, Google, Oracle Health, Epic, and 55+ other organizations.
Pledged categories: **Conversational AI, Kill the Clipboard, Chronic Care.**

THREE DISTRIBUTION CHANNELS · ONE APP



Medicare App Library

CMS-promoted directory · InPursuit lists at app launch

67M

BENEFICIARIES



Apple App Store

Universal consumer reach · iOS

D2C

ALL PAYERS



Google Play

Universal consumer reach · Android

D2C

ALL PAYERS

WHY MEDICARE IS THE RIGHT COHORT

Highest chronic disease prevalence

— 95% have 1+ chronic condition, 85% have multiple

Already conditioned

to digital health tools and remote monitoring

Highest device ownership

in the population — the Reflect hub thesis at scale

Concierge converts higher here

— clinical access is the actual unmet need

INPURSUIT HEALTH · CONFIDENTIAL [260806.2]

THE ADVANTAGE

Most pitches project 0.25% penetration. We have the distribution channel that most pitches don't.

COMBINED EXIT ARR · MID-2027

DISTRIBUTION · CMS FIRST WAVE · 67M REACH · \$3.67M D2C · \$257K ENGINE 2 UPLIFT

16 / 31

CONSERVATIVE 12-MONTH PROJECTION

0.25% penetration into the Medicare App Library cohort — *the floor most digital-health pitches use.*

168,500

MEMBERS BY MID-2027

TIER	SHARE	MEMBERS	ARPU	MONTHLY
Free <i>top of funnel</i>	95.0%	159,125	\$0	—
Plus <i>Reflect hub</i>	3.5%	5,863	\$14.99	\$87.9K
Family <i>household</i>	0.5%	838	\$34.99	\$29.3K
Total Intelligence <i>individual flagship</i>	0.7%	1,173	\$49.99	\$58.6K
Total Family Intelligence <i>household flagship · additive</i>	+1,000 households	1,000	\$89.99	\$90.0K
TI + Concierge <i>full clinical stack</i>	0.3%	503	\$78.99	\$39.7K
D2C ARR EXIT RUN-RATE		168,500	\$3.67M	

+ ENGINE 2 CONVERSION UPLIFT

\$257K ARR

503 Concierge members x \$42.50/mo blended Engine 2 conversion (GCM, RPM) — billed through IPMC, independent of subscription fee.

COMBINED EXIT ARR · MID-2027

\$3.93M

ENGINE 02



The InPursuit
Partner Network.

Results Matter™

INPURSUIT PARTNER NETWORK • ENGINE 02 INTRODUCTION

A *membership* for the practices that built American medicine.

WHAT MEMBERSHIP DELIVERS

Membership in the InPursuit Partner Network provides the 230,000 small & mid-size providers with the ability to finally deliver digital care, modern technology, and multi-payer revenue — *without building it alone.*

THREE LAYERS • FIFTEEN SERVICE LINES • FUNDED BY CARE, NOT FEES

01

The *operating* layer

Eligibility and enrollment, documentation and billing, and a **care team staffed for the practice**. There is no one to hire, nothing to buy, no implementation to survive.

STAFFED • ALREADY RUNNING

02

The *patient* layer

The app, the unified record, and between-visit engagement, **live across the whole panel** — with FDA-cleared vitals captured from the phone the patient already owns.

PANEL-WIDE • EVERY PAYER

03

The *care engine*

Fifteen service lines — cardiometabolic, respiratory, renal, behavioral, transitions — turn on one, add the rest with **no new contract and no new integration**.

ONE CONTRACT • ONE INTEGRATION

04

Delivery, *their way*

Full Handoff: InPursuit delivers and bills every digital line. **Split:** already run a line? Keep it, bill it, keep 100% — InPursuit delivers the rest. Chosen line by line.

TWO MODELS • ONE MEMBERSHIP

05

Independence, preserved

Practice keeps the patient relationship. Practice keeps 100% of existing FFS. **Not an Aledade. Not a health system buyout.** A membership.

ADDITIVE • NOT EXTRACTIVE

FOUNDING MEMBER

Honeycomb Medical Group

Memphis, TN • eClinicalWorks EMR

The first practice to join the network. Active deployment. Five InPursuit revenue layers in motion — made possible by Honeycomb's panel.

WHAT MEMBERSHIP LOOKS LIKE IN PRACTICE — NEXT SLIDE.

22,000

PATIENT
PANEL

5

REVENUE
LAYERS

Multi

PAYER
MIX

THE INPURSUIT NETWORK • WHY IT MATTERS

The care your patients need — *that you can't deliver alone today.*

Chronic care management, remote monitoring, coordinated outcomes, access to CMS innovation models — proven to keep patients healthier and out of the hospital. Most small and mid-size practices can't stand them up alone, so their sickest patients go without.

CCM • CHRONIC CARE MGMT

~4%

of eligible Medicare patients actually receive chronic care management.

Roughly 12,000 of the nation's ~1 million Medicare providers billed CCM — the capability barely exists where patients live.

Two in three Medicare patients live with multiple chronic conditions. **Almost none of them are enrolled.**

MATHEMATICA / KFF HEALTH NEWS, 2024

RPM • REMOTE MONITORING

<2%

of Medicare beneficiaries are enrolled in remote patient monitoring.

RPM reached roughly 1 million of ~68 million beneficiaries in 2024 — concentrated in a handful of practices, mostly for diabetes and hypertension.

The devices and reimbursement exist. **The staffing to run it daily doesn't — not in a small practice.**

HHS OIG DATA SNAPSHOT, 2025

APCM • ADVANCED PRIMARY CARE

13 elements

CMS now pays a practice monthly to make 13 care elements *always available*.

24/7 access, care coordination, a patient-centered plan, risk stratification, population analytics — not visits, but an always-on care system behind the patient.

Those 13 elements *are* infrastructure. **InPursuit is how a small practice actually delivers them.**

CMS PHYSICIAN FEE SCHEDULE, 2025-2026

WHAT INPURSUIT DELIVERS	CCM	RPM	APCM
The care, stood up and staffed for you.	Chronic Care Management Monthly coordination for patients with multiple conditions — run by InPursuit's care team.	Remote Patient Monitoring FDA-cleared vitals from the phone the patient already owns — daily readings and clinical response without adding staff to your front desk.	Advanced Primary Care Mgmt The 13 always-available service elements CMS requires — delivered as managed infrastructure.

THE GAP TODAY This isn't lost revenue — it's **care your patients aren't getting**, and the funding that would sustain it left on the table. A small practice can't build the staffing, technology, and billing infrastructure alone. **Membership is how it finally can.**

THE INPURSUIT NETWORK • HOW A PRACTICE JOINS

Two membership tiers. *One orchestration layer* underneath.

Every IPN membership runs on a fair-market-value management agreement and InPursuit's infrastructure. Practices choose the depth of deployment: core network access, or a fully branded experience that makes InPursuit's infrastructure their own.

TIER 01 • NETWORK MEMBER • INPURSUIT-FUNDED

InPursuit Growth

Everything needed to deliver the chronic-care, monitoring, and coordinated-outcome services your patients need today — and can't get from you alone.

- ✓ **InPursuit infrastructure** — data orchestration, care coordination, outcome reporting
- ✓ **Full care-management capability** — CCM, RPM, APCM, BHI, delivered by IPMC
- ✓ **The InPursuit app** for the practice's patients
- ✓ **Billed by IPMC under its own NPI** — practice refers, IPMC delivers and bills

The practice operates **within the InPursuit Network** — patients experience the InPursuit brand. Ideal for practices that want the care delivered without the branding lift.

TIER 02 • BRANDED DEPLOYMENT • \$1/PATIENT/YR

POWERED-BY

InPursuit Signature

Everything in Growth — plus the practice deploys the entire infrastructure under **its own brand**.

- ✓ **Everything in Growth**, included
- ✓ **Fully branded patient app & portal** — the practice's name, the practice's identity
- ✓ **Branded care experience** end-to-end, powered by the InPursuit infrastructure underneath
- ✓ **Patient loyalty stays with the practice** — not a third-party app

E.G. "Honeycomb, Powered by InPursuit"

HOW IT'S STRUCTURED

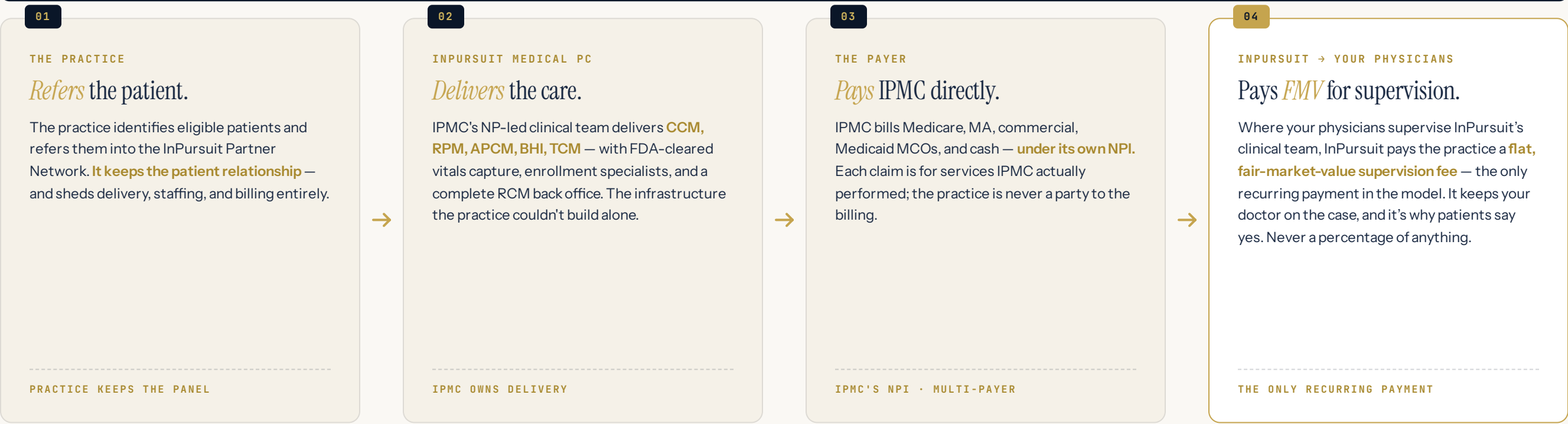
Both tiers operate under a **fair-market-value Management Services Agreement**, structured to each practice's state CPOM and anti-kickback requirements. **There is no financial relationship between InPursuit and the practice** — InPursuit Medical PC delivers the clinical services and bills payers under its own NPI; the practice refers and keeps the relationship.

THE INPURSUIT NETWORK · HOW CARE GETS DELIVERED

Practice refers. *IPMC delivers and bills.*

Every IPN member runs on the same model: the practice refers patients into the digital services it can't deliver alone, and InPursuit Medical PC (IPMC) takes care of everything else — delivery, billing, and staffing — under its own NPI. No fee for care delivery ever flows from the practice to InPursuit.

EVERY PATIENT GETS The *same* coordinated care — chronic care, monitoring, outcomes. SAME MODEL, EVERY MEMBER



ANTI-KICKBACK · BY DESIGN

The practice never pays InPursuit for care delivery — no fee, no percentage of collections or savings. IPMC bills its lines under its own NPI; the practice bills its own work and keeps 100%. The only recurring payment is a flat fair-market-value supervision fee to partner physicians for oversight actually performed — structured to each state's CPOM and the AKS personal-services safe harbor, 42 CFR §1001.952(d). Every partner in this market sends the practice an invoice; we send the practice a check.

ENGINE 02 • PROOF OF REAL CUSTOMER

The care Honeycomb's 22,000 patients finally get.
230,000 practices need the same.



ANCHOR PRACTICE
 FOUNDING MEMBER • IPN

Honeycomb Medical Group

MEMPHIS, TENNESSEE • ECLINICALWORKS EMR

22,000

FULL PATIENT PANEL • MULTI-PAYER

Funds ~\$2.08M / yr of care by Year 3

Anchor-scale — a 22,000-patient practice, not a typical member. Conservative floor.
 Commitment signed; onboarding begins within 4 weeks of close — the proof Engine 2 is
 real before a dollar of the round is spent.

THE PANEL

~7,700 RPM-eligible • ~5,500 CCM-eligible • ~3,300 BHI-eligible

Across every payer type. **Patients Honeycomb structurally cannot serve with clinical-services programs alone.** InPursuit delivers infrastructure; Honeycomb refers and keeps the panel.

THE MODEL

No SaaS fee. No per-patient fee. *No percentage of revenue.*

Honeycomb never pays a fee for care delivery — never a percentage of collections or savings. InPursuit Medical PC delivers CCM, RPM, APCM, BHI, TCM and bills payers directly under its own NPI — across Medicare, MA, commercial, Medicaid MCOs, and cash. Honeycomb refers patients, keeps the relationship, and sheds delivery, staffing, and billing entirely.

Honeycomb is the proof point — not the ceiling.

THE FIRST MEMBER • HONEYCOMB MEDICAL GROUP • 22,000 PATIENTS

Care Honeycomb couldn't deliver alone — *now it can.*

Chronic care management, remote monitoring, coordinated outcomes — care a 22,000-patient practice can't stand up on its own. Membership makes it possible, and makes it pay for itself. The revenue below funds the care; it isn't the reason to join. Honeycomb refers the patient; InPursuit Medical PC delivers and bills.

CCM

Chronic Care Management

Monthly coordination for Honeycomb's multi-condition patients.

RPM

Remote Patient Monitoring

FDA-cleared vitals from the patient's own phone, daily clinical response, fully staffed.

APCM

Advanced Primary Care

The 13 always-available care elements, delivered as infrastructure.

D2C

Consumer Membership

The InPursuit app pulls patients into their own care.

WHAT MEMBERSHIP DELIVERS

Every Honeycomb patient gets coordinated chronic care, monitoring, and outcome tracking the practice **couldn't deliver alone**. The revenue is what makes that care sustainable — it isn't the reason to join.

Honeycomb refers; IPMC delivers and bills under its own NPI. Honeycomb never pays a fee for care delivery. Figures are Y3, conservative floor.

THE MODEL • INPURSUIT MEDICAL PC

Refer the care. Same outcome.

InPursuit Medical PC delivers and bills under its own NPI. Honeycomb keeps the relationship, sheds admin, staffing & billing.

INPURSUIT • Y3

~\$2.08M

Zero cost, zero admin to Honeycomb

THE INPURSUIT NETWORK • THE PER-MEMBER BASIS

One per-member basis. *Live proof + conservative projection.*

Honeycomb is the live proof point, modeled conservatively at a ~10K-equivalent panel for projections — its real 22K is upside, not the figure we build the model on. Every other member is modeled on the same conservative ~10K basis. The ladder underneath stacks identical per-member units.

LIVE PROOF • FOUNDING MEMBER

Honeycomb

22,000

PATIENTS • LIVE MULTI-PAYER

LIVE STATUS

Founding member, commitment signed — the proof Engine 2 is real before a dollar of the round is spent

IN THE MODEL

Carried at ~10K-equivalent (~\$1.25M Y3) — the same basis as every other member

UPSIDE

True 22K run-rate as a Full Handoff split ~\$2.08M Y3 — held out of the ladder; the gap to the carried unit is upside

Real customer, conservative modeling — the upside lives in the 22K-vs-10K gap.

THE PROJECTION BASIS • PER-MEMBER UNIT

~10K-panel member

~\$1.25M

Y3 RUN-RATE • FULL HANDOFF

DEFINITION

A small/mid-size practice with a ~10,000-patient multi-payer panel — roughly half of Honeycomb's true scale

THE MODEL

Refer the patient — IPMC delivers. Practice builds and risks nothing

WHY THIS BASIS

Conservative Full Handoff basis; the per-member unit is ~\$1.25M, carried flat across the ladder

This unit stacks. The Y3 ladder multiplies this figure across the network.

THE TAKEAWAY

Honeycomb is the live proof. The ~10K-equivalent unit is the basis — every member, Honeycomb included, is modeled on it. The next slide stacks this unit across 5 / 15 / 25 members. Median CCM biller manages ~10 patients; 42% are solo (CMS). Outsourcing removes the in-house staffing barrier.

GRANT-FUNDED ACQUISITION • NON-DILUTIVE

A \$50 billion federal window is open now. *We’ve already filed.*

Rural Health Transformation funds a rural practice’s entire InPursuit infrastructure — the state pays, not the practice. Non-dilutive customer acquisition, and the core of our rural sales motion.

\$50B

5-YEAR FEDERAL PROGRAM

\$10B / yr

AVAILABLE THROUGH 2030

~\$200M

ALREADY AWARDED • FY2026

50 states

CHOOSING PARTNERS NOW

1

The state pays

A rural practice gets InPursuit’s infrastructure funded by Rural Health Transformation dollars — not out of its own pocket.

2

The practice gains the capability

Full care-management infrastructure stood up, and InPursuit is paid at onboarding.

3

The template repeats

Across every rural member, in any of 50 states, while the states are deciding who to spend the money with right now.

ALREADY FILED

\$300K submitted for Honeycomb’s Mississippi footprint

\$165K technology backbone + \$135K vitals-verified telehealth · ~2,000-patient footprint · awards projected August 2026

The playbook, already proven once — ready to repeat across 50 states.

REVENUE PROJECTION



Engine 01 +
Engine 02.

ENGINE 01 • D2C *Harness the Power of Your Data™*

ENGINE 02 • IPN *Results Matter™*

TWO ENGINES • TWO UNIT-ECONOMIC PROFILES

Two engines, two paths to *contribution*.

ENGINE 01 • PER PAYING MEMBER • ANNUAL

A *Total Intelligence* member at \$49.99/mo.

REV	Membership fee • \$49.99/mo	<i>\$600</i>
c2s	Infra, device API, AI inference, content	<i>(\$131)</i>
c2s	App Store fees • 15% blended	<i>(\$90)</i>
CAC	Panel-introduced via IPN • direct D2C modeled at \$120	<i>\$0</i>
CM	Annual contribution per Total Intelligence member	<i>~\$379</i>

Concierge add-on (**\$29/mo, TI-gated**) opens the Engine 2 conversion lane. Bundle contribution including Engine 2 care-management uplift: **~\$50/mo** per Concierge member • clinical churn modeled conservatively at 4–7%/mo. Panel-channel member shown; direct D2C carries the \$120 CAC.

ENGINE 02 • PER ~10K-PANEL MEMBER • YEAR 3 RUN-RATE

A *~10K-panel* member at full ramp.

REV	Medicare, MA & commercial parity • CCM/RPM/APCM/BHI • billed under IPMC's own NPI	<i>\$1.25M</i>
c2s	Clinical delivery (NP team), FDA-cleared devices, enrollment ops	<i>(\$0.45M)</i>
c2s	RCM, credentialing, infrastructure & AI	<i>(\$0.13M)</i>
CM	Contribution per ~10K-panel member • ~53%	<i>~\$0.67M</i>

Billed under **IPMC's own NPI** across Medicare, MA & commercial — **no revenue share, no percentage to the practice** (AKS personal-services safe harbor, 42 CFR §1001.952(d)). Conservative ~10K-panel basis; Honeycombs true 22K panel **held as upside**. Cost lines illustrative pending final clinical model.

YEAR 3 • FULL HANDOFF BASIS • COMBINED (ENGINE 1 + ENGINE 2)

Two engines. *One per-member basis.* One cap table.

YEAR 3 • ANCHOR • 25 MEMBERS

~\$35M *combined y3*

25 IPN members, each modeled at ~10K-equivalent panel · Engine 1 held flat at Y1 exit ARR · Full Handoff basis · CIN and D2C scaling held as upside.

E2	~\$31.2M	25 members × ~\$1.25M per ~10K-panel member · IPMC bills under its own NPI · Honeycomb carried at conservative ~10K-equivalent
E1	~\$3.67M	D2C membership held flat at Y1 exit ARR · 167,500 base members + 1,000 Total Family Intelligence households (TFI additive)
E2+E1	~\$35M	Combined Year-3 run-rate · conservative basis · per 260806.2 reconciled across exec summary, IPN, and D2C source docs

THE Y3 LADDER • THIS UNIT STACKS

Each rung = (members × ~\$1.25M Engine 2) + ~\$3.67M Engine 1 flat. Conservative basis throughout.

5 MEMBERS	~\$9.9M
15 MEMBERS	~\$22M
25 MEMBERS • ANCHOR	~\$35M
50 MEMBERS	~\$66M
100 MEMBERS	~\$128M

UPSIDE EXCLUDED: CIN, HONEYCOMB TRUE 22K, D2C SCALING

FOUNDERS & LEADERSHIP

The team that is *InPursuit of building* a Person-Centric Healthcare Ecosystem.

FOUNDERS



Bennett McPhatter

USMC VETERAN

Co-Founder & CEO

30+ year career building distributed intelligence and data platforms across the U.S. Department of Defense, intelligence community, financial services, healthcare, and the Fortune 500. Former Intelligence NCO. Successfully built, grew to 32 countries, and sold Visual Analytics to Raytheon.



Hongda Lin

Co-Founder & CTO

25+ years building federated search and large-scale data analytics systems for U.S. intelligence and law enforcement agencies. Architected high-throughput data orchestration deployed across federal healthcare and national security programs. MS Computer Science, University of Maryland.



Chris Ingram

Co-Founder & CPO

Built the neural network predictive engine powering InPursuit's forecasting layer. Former Vice President at MSCI, where he led quantitative product development for global institutional investors. BS Computer Engineering, MBA. Deep expertise in machine learning, time-series modeling, and enterprise solutions.



Patrick Truxillo

Co-Founder & CSO

Data integration specialist and healthcare entrepreneur. 20-year career building open-standards solutions that connect disparate data sources into usable insight — spanning DoD interoperability programs (including MHS GENESIS and VA FENRM), federal law enforcement, financial services, and government.

EXECUTIVE TEAM



Dr. Joseph M. Chalil

U.S. NAVY VETERAN

Chief Medical Officer

20+ years health policy. Three U.S. patents. IAOTP Top Physician of the Year.



Jose Arrieta

Chief Innovation Officer

Former U.S. Federal Chief Data Officer. Blockchain supply chain pioneer. National data modernization leader.



Michael Mark

AIR FORCE VETERAN

CRO — VBC

Former healthcare provider and payer CEO with decades building healthcare partnerships. Leads InPursuit Partner Network go-to-market strategy.



Alison Cossette

Chief AI Officer

Responsible AI expert. Graph data science, provenance systems, trustworthy AI architecture.

CONVICTION CAPITAL

Executive team is currently engaged on a fractional, uncompensated basis in conviction capital. Founders are compensated at close; the broader executive team remains fractional until revenue supports full comp.

STRATEGIC PARTNERS & ADVISORY

Operators, distribution, and counsel *behind the build.*



STRATEGIC PARTNER

HEALTHCARE GPO & DISTRIBUTION NETWORK • 0.05% EQUITY

A strategic partner, incubator, and vested equity stakeholder. Through its group-purchasing and distribution network, WellLink is bringing InPursuit’s membership and solutions to **hundreds of thousands of healthcare providers across the country**. CEO **Brian Lane (FACHE, FHIMSS)** serves as InPursuit’s **appointed Advisory Board representative**.

“WellLink is excited to be an incubator and advisory partner of InPursuit’s innovative healthcare solutions. The lack of interoperability has created a disjointed healthcare system, riddled with inefficiencies that can delay critical care, drive up costs, and compromise patient outcomes. We are excited to be part of shaping and delivering a better healthcare ecosystem with InPursuit Health.”

— BRIAN LANE, CEO, WELLLINK

ADVISORY BOARD



Alexis Johnson

CLINICAL LEADERSHIP & OPERATIONS

MSN, Health Leadership. Former Regional Director of Operations across 10+ hospitals (~900K annual visits, ~\$50M EBITDA); built multi-state readmission-reduction programs. National Director of Integrations, US Health Partners.



Brian Lane

WELLLINK CEO • APPOINTED REP

FACHE, FHIMSS. Healthcare GPO and distribution-network leader connecting provider organizations and enterprise partners nationwide. Brings distribution reach and enterprise health-system credibility to InPursuit’s go-to-market.



David M. Janet

LEGAL, IP & COMPLIANCE

Member, Fletcher, Heald & Hildreth. Duke Law. 20+ years in healthcare technology transactions, IP licensing, HIPAA, data-integration agreements, and anti-kickback / regulatory structuring for health-tech companies.

THE INVESTMENT THESIS · WHAT \$3.5M ACTUALLY BUYS

\$3.5M seed. App live in weeks. *Both engines by Q4.*

To hit the Year-3 anchor of ~\$35M combined, *InPursuit needs* twenty-four more members like Honeycomb. *Not 24,000. Not 2,400. Twenty-four.* The technology is built, the compliance posture is in place, and Honeycomb is signed. What \$3.5M funds is not technology development — it's the launch that turns built infrastructure into recurring revenue across both engines.

NOW · THE SEED ROUND

\$3.5M

Class C Units

NEW UNITS · PRICED EQUITY

\$20M Pre-Money

\$23.5M POST-MONEY

STRUCTURE

Priced equity

MINIMUM CHECK

\$25K

CLOSE

Single close

CONVERSION

C-Corp · QSBS

Class C units convert at the C-Corp conversion, where QSBS eligibility begins. Next round optional, not required — operations increasingly carried by collections.

USE OF PROCEEDS

Everything starts on day one. *Nothing waits on anything else.*

~\$100K

The app to the stores · live 4–6 weeks after the first \$100K clears

~\$900K

Clinical services stand-up · staffed ahead of collections

~\$350K

Partner delivery capacity · Engine 2 on · Honeycomb live

~\$900K

Team & production · key hires, cloud & AI, payer integrations

~\$300K

Member acquisition & marketing · ignites at launch

~\$400K

Corporate foundation · legal, insurance, compliance, finance ops

~\$550K

Operating reserve & working capital · bridging the collections lag

DOCUSIGN · SINGLE CLOSE · \$3.5M TOTAL

INVEST@INPURSUITHEALTH.COM

"What \$3.5M actually buys: ownership in the infrastructure for a person-centric healthcare world. And the capital to scale from one Honeycomb to 25 providers on infrastructure designed for unlimited scale."

INVEST@INPURSUITHEALTH.COM
HARNESS THE POWER OF YOUR DATA™ · RESULTS MATTER™

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31 / 31